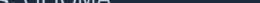


Report ID:	RPT-A8796210	Generated:	2026-04-20 15:46:30 IST
Patient:	Anonymous	Study Date:	April 20, 2026
Patient ID:	N/A	Modality:	MRI Brain (AI Inferred)
Model:	BrainTumorAI v2.0 Ensemble	Model AUC:	0.9996 Acc 98.78%

PRIMARY FINDING

GLIOM A

Diagnosis: GLIOMA
Confidence:  81.1%
Risk Level:  CRITICAL
Processing: 8.05s (TTA 6-view ensemble)
Method: EfficientNet-B3 + ResNet50+CBAM + DenseNet121

DIFFERENTIAL DIAGNOSIS

[illegible]

ENSEMBLE AGREEMENT

Model	Confidence	Bar
EfficientNet-B3	96.1%	
ResNet50+CBAM	47.5%	
DenseNet121	94.7%	
Ensemble (TTA)	81.1%	← FINAL

CLINICAL INTERPRETATION

The AI model identifies imaging features consistent with a glial neoplasm. The Grad-CAM localisation map highlights a region of interest with high activation intensity. Gliomas represent the most common primary brain tumours and vary widely in grade (WHO I–IV). Immediate referral to neurosurgery and neuro-oncology is strongly recommended. MRI with contrast enhancement, spectroscopy, and perfusion imaging should be obtained. Biopsy and molecular profiling (IDH status, MGMT methylation) may be required for definitive grading.

RECOMMENDATIONS

1. Urgent MRI brain with gadolinium contrast enhancement
2. Neurosurgical consultation within 48 hours
3. Neuro-oncology multidisciplinary team review
4. Biopsy / resection planning (IDH, MGMT, EGFR profiling)
5. Baseline neuropsychological assessment

■ **DISCLAIMER:** This report is generated by NeuroScan AI, an AI-assisted screening tool. It is intended for informational and research purposes only and does **NOT** constitute a medical diagnosis. All findings must be reviewed and confirmed by a qualified radiologist or neurologist before any clinical decision is made. Individual model performance may vary across patient populations and imaging protocols. Reported metrics: AUC 0.9996, Accuracy 98.78%, Sensitivity 98.68%, Specificity 99.59% (test set n=1,311 images). Report generated: 2026-04-20 15:46:30 IST | Report ID: RPT-A8796210